

Diagnosis of heart failure in primary care: missed opportunities



Although there have been major advances in the development of effective treatments for heart failure, 5-year survival is still only around 50%.¹ Prognosis is particularly poor in people who are first diagnosed after emergency admission to hospital.² Therefore, earlier diagnosis could potentially lead to improved outcomes. As a result, there is value in studies assessing whether symptoms of heart failure occurred before diagnosis, and how these symptoms were investigated.

In an Article published in *The Lancet Primary Care*, Claire A Lawson and colleagues³ report the results of a retrospective cohort study using electronic health records of adults diagnosed with heart failure in England between 2000 and 2021. They examined whether the diagnosis was made as an outpatient or an inpatient and whether there were indicators of heart failure in the primary care records (eg, breathlessness, ankle swelling, and loop diuretic use) in the 5 years before diagnosis. They also assessed diagnostic investigations (eg, natriuretic peptide tests, echocardiography, and specialist review) conducted within 6 months before diagnosis. Moreover, they explored the associations of these diagnostic pathways with survival.

181 287 (44·0%) of 412 173 diagnoses were made after emergency hospital admission in the inpatient setting, and the survival of patients diagnosed at this time remained substantially worse than those diagnosed as an outpatient. Patients admitted to hospital had symptoms suggestive of heart failure recorded in their health records on average more than 2 years earlier. 175 041 (63·8%) of 274 228 people with previous symptoms or diuretic use had none of the recommended investigations conducted in primary care before diagnosis. Women, people living in areas of socioeconomic deprivation, and people with multimorbidity were less likely to be investigated or diagnosed as an outpatient.

The focus on people with a final diagnosis of heart failure only gives a partial picture—we do not know how many people with suggestive symptoms who did not have heart failure were investigated. The analysis relied on electronic coding of heart failure diagnoses, which can be unreliable.⁴ Nevertheless, these limitations do not undermine the overall message: diagnosis of heart failure

in primary care takes too long, does not follow guideline-recommended approaches, and accentuates inequities in health.

Symptoms of heart failure are non-specific,⁵ so earlier investigation of all people with such symptoms is not the answer. Primary care needs better tools to identify who is at risk of heart failure and to diagnose the condition.⁶ Investigation of people started on diuretic therapy is important given that people on diuretic therapy without a label of heart failure have a similar prognosis to those that do have such a label, suggesting substantial under-diagnosis.⁷ Multimorbidity was associated with delays in diagnosis, suggesting the index of suspicion might be higher in people with more than one chronic condition. The authors rightly observe that referrals for diagnostic testing are hindered by administrative constraints such as rigid referral protocols, limited access to diagnostic services, and prolonged waiting times for specialist assessments.⁸ These challenges are further compounded by guidelines for management of heart failure, which indirectly position secondary or tertiary care (eg, cardiologists) as the primary setting for heart failure diagnosis. Consequently, patients are often routed to secondary care for advanced testing, resulting in substantial delays. In some regions, access to echocardiography services can take up to 12 months, delaying treatment optimisation and worsening patient outcomes. Therefore, more nuanced solutions are required than better adherence to guidelines only, as better adherence might have the unintended consequence of increasing referral volumes and putting further strain on an already overstretched secondary care system. The question of how to reduce inequities in delivery of care for people with symptoms of heart failure is one that extends beyond the confines of diagnosis of heart failure; consideration of barriers to health-care services and the ways people are managed when they do access these services are of particular relevance in this context.⁹

Lawson and colleagues³ provide information on diagnosis of heart failure in the UK in the 21st century. A salutary lesson is that diagnostic delays and the proportion of people first diagnosed during an emergency admission to hospital are no better now than at the turn



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of the century. Potential solutions can be informed by research to develop better risk assessment and diagnostic tools, and by consideration of pathways for diagnosis that consider system constraints and reasons for inequities in accessing care services, as well as subsequent management.

We declare no competing interests.

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